

Reading the Scalp

The consultation, common conditions, and knowing precisely where your practice ends

Learning Objectives

- Conduct a visual and tactile scalp assessment before any service begins
- Identify the common scalp and hair conditions a practitioner will regularly encounter
- Distinguish conditions that are within a practitioner's scope from those that require referral
- Explain why referring a client to a doctor or trichologist is a mark of professionalism, not failure

Look, and ask, before you touch

No service at Amber — cut, colour, or style — begins without a scalp and hair assessment. This is not a formality. It is the moment you decide whether the service you were about to perform is even appropriate for the hair and scalp in front of you, and it is the moment you catch conditions a client herself may not have noticed or named.

A practitioner who assesses before she touches is protecting her client. A practitioner who touches before she assesses is only hoping for the best.

The four-part scalp assessment

1. **Visual scan.** Part the hair in several sections and look directly at the scalp for redness, flaking, sores, bald patches, or unusual texture.
2. **Tactile check.** Gently palpate the scalp for tenderness, unusual bumps, or areas that feel different in temperature or texture from the surrounding skin.

3. **Strand examination.** Assess the hair itself — texture, porosity, breakage, and any visible thinning along the parting or hairline.
4. **Verbal history.** Ask about recent chemical services, medication changes, stress, illness, diet, and any family history of hair loss. Hair often tells a story about the body that the client has not connected to her hair at all.

Conditions within a practitioner's scope

- **Dandruff (pityriasis capitis).** Flaking, often with mild itch, caused by an overgrowth of the yeast *Malassezia*. Manageable with anti-fungal shampoos and scalp care; very common and not a sign of poor hygiene.
- **Dry, dehydrated scalp.** Tightness and fine flaking without the oiliness typical of dandruff. Responds well to gentle cleansing and scalp oils.
- **Oily scalp / seborrhea.** Overactive sebaceous glands producing excess oil, often worsened by over-washing or harsh stripping shampoos that trigger rebound oil production.
- **Mechanical breakage.** Snapped strands from tension, heat, or chemical damage, distinct from hair loss at the root — a distinction you will learn to make with confidence below.

Conditions that require referral

A practitioner's scope ends where diagnosis and medical treatment begin. The following signs call for a respectful, unhurried conversation and a referral to a dermatologist or trichologist — never a diagnosis delivered by you, and never a service performed to "fix" a condition you are not trained to treat.

- **Sudden or patchy hair loss** (alopecia areata), which appears as smooth, coin-sized bald patches and has an autoimmune basis.
- **Diffuse thinning across the whole scalp**, which may signal androgenetic alopecia, thyroid imbalance, iron deficiency, or postpartum shedding — covered further in Module Eight.
- **Scaling, crusting, or open sores**, which may indicate psoriasis, eczema, fungal infection (tinea capitis), or folliculitis.
- **Scarring alopecia**, in which the scalp appears smooth and shiny with no visible follicle openings — a sign of permanent follicle damage requiring urgent medical attention, most

often seen in advanced traction alopecia or certain autoimmune conditions.

Explaining scope of practice to a client is never a moment of failure. Say plainly: "This looks like something a dermatologist should examine properly before we go further — I want you looked after well, not just serviced today." A client remembers that sentence for years, and she remembers who said it to her.

A Note from Practice

A client arrives for a routine trim and mentions, almost in passing, that her ponytail has felt "thinner lately, but it's probably just the weather." A visual scan shows smooth, shiny patches along her hairline with no stubble regrowth — a sign consistent with traction alopecia from years of tight braiding.

The correct response is not to proceed with the trim as booked and say nothing. It is to gently name what you see, explain that this pattern is often reversible if caught early but needs a specialist's guidance, and adjust today's service to protect the hairline rather than stress it further — loosening any tension, avoiding tight styling, and documenting the referral conversation in her consultation card.

Key Terms

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[object Object]

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Practical Activity · The Assessment Card

Perform a full four-part scalp and hair assessment on a partner or model. Complete a written consultation card recording your findings. If you observe nothing requiring referral, note that too — an assessment that finds nothing of concern is still a complete assessment. **File this card; it is your first completed consultation record for Module Ten.**